

SOP-008

Adverse Events, Protocol Deviations, Protocol Exceptions, Unanticipated Problems, and Non-Compliance

Standard Operating Procedure | Template | Version 1.1

Site Name:	[Site Name]	Effective Date:	[Enter Date]
Prepared By:	[Enter Name]	Approved By:	[Enter Name]
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TEMPLATE — Replace all [bracketed placeholders] before use

Principal Investigator Approval

Signature: _____ Print Name: _____ Date: ____ / ____ / ____

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1. Objective

The objective of this Standard Operating Procedure (SOP) is to describe the methods to resolve and report adverse events (AEs), protocol deviations, protocol exceptions, unanticipated problems, and non-compliance that may occur during a clinical trial.

2. Scope

This SOP applies to all clinical research staff involved in the conduct of clinical trials at [Site Name].

3. Responsibilities

3.1 Principal Investigator (PI)

The PI is responsible for accurate and timely reporting of adverse events (AEs) to the Sponsor and Institutional Review Board (IRB) (if applicable).

The PI is also responsible for reporting any serious adverse events (SAEs), protocol deviations, unanticipated problems, and non-compliance to the Sponsor and IRB within their specified time frames and according to applicable policies and regulatory requirements.

3.2 Investigators and Clinical Research Coordinators (CRCs)

All Investigators and CRCs are responsible for:

- Recording all new clinical experiences, exacerbations, and/or deterioration of any existing clinical condition occurring after a study participant has been enrolled in the study, on the appropriate case report forms.
 - Providing follow-up information on all adverse events until resolution, or an appropriate endpoint is reached.
 - Reporting any serious adverse events, protocol deviations, unanticipated problems, and non-compliance to the PI.
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4. Definitions

4.1 Adverse Event (AE)

Per ICH E6(R2) Section 1.2: Any untoward medical occurrence in a participant or clinical investigation participant administered a pharmaceutical product and that does not necessarily have a causal relationship with this treatment. An AE can therefore be any unfavorable and unintended sign, symptom, or disease temporally associated with the use of a medicinal (investigational) product, whether or not related to the medicinal product.

ICH E6(R3) Note: ICH E6(R3) is the current version of the GCP guideline. E6(R3) introduces Quality by Design (QbD) principles, requiring that quality management systems be proportionate to the risks inherent in the trial and the importance of the information collected. Sites should ensure their AE management processes align with proportionality-based risk assessment and that Critical to Quality (CtQ) factors related to participant safety are identified prospectively.

4.2 Serious Adverse Event (SAE)

Per ICH E6(R2) Section 1.50: Any untoward medical occurrence that at any dose:

- Results in death
- Is life-threatening
- Requires or prolongs inpatient hospitalization
- Results in persistent or significant disability/incapacity
- Is a congenital anomaly or birth defect

Important medical events that may not result in death, be life-threatening, or require hospitalization may be considered a serious adverse event when, based upon appropriate medical judgment, they may jeopardize the participant and may require medical or surgical intervention to prevent one of the outcomes listed above.

4.3 Unanticipated Problems Involving Risk to Participants or Others

Any problem, event, or new information that:

- Is unexpected (in terms of nature, severity, or frequency) given the research procedures described in the protocol-related documents, such as the IRB-approved research protocol, drug/device labeling, and informed consent documents, and the characteristics of the participant population being studied; AND
- Is related or possibly related to participation in the research; AND
- Indicates that participants or others are at a greater risk of harm (including physical, psychological, economic, or social harm) than was previously known or recognized.

5. Procedures

5.1 Adverse Events

After a study participant has enrolled in a clinical study, any new clinical experience, exacerbation, and/or deterioration of any existing clinical condition should be recorded in the study participant's study binder and then transcribed onto the Adverse Event CRF (if applicable) or directly into the Electronic Data Capture (EDC) system.

The CRC is responsible for reviewing with the participant and Investigator all adverse event information during study participant visits. The CRC is also responsible for the following:

1. Recording all new clinical experiences, exacerbations, and/or deterioration of any existing clinical condition occurring after a study participant has entered the study in the participant's study binder and on the appropriate CRF or EDC.
2. Notifying the PI immediately when ANY adverse event is reported. The PI evaluates the medical condition, manages medically as appropriate, records the information in the medical record, and the information is then entered into the participant's binder with corresponding source documents and

submitted to the Sponsor or IRB as indicated per protocol.

3. If the event is serious: It is reported to the PI immediately for review. After review and approval by the PI, the CRC reports the SAE to the Sponsor **per the protocol's specified SAE reporting timeline** (typically within 24 hours of site awareness, consistent with the investigator's obligation to report serious adverse events to the sponsor "immediately" per **ICH E6(R2) §4.11.1**) and to the IRB per IRB-specific reporting requirements (typically within 24 hours for events that are both serious and unexpected, or per the IRB's written policies — frequently anchored to **45 CFR 46.103(b)(5)** / **21 CFR 56.108(b)(1)** for unanticipated problems involving risks to participants). Follow-up reports and timelines for fatal or life-threatening events shall follow the protocol's specified cascade.

Regulatory note (E6(R2) §4.11.1 / §4.11.2): Investigator-to-sponsor SAE reporting is governed by the **protocol** and **ICH E6(R2)**, not directly by 21 CFR 312.32. **21 CFR 312.32 governs the sponsor's obligation to file IND safety reports with the FDA** (initial 7- and 15-day cascade for fatal/life-threatening or unexpected suspected adverse reactions). Sites should report to the sponsor on the protocol-specified timeline so the sponsor can meet its 312.32 obligations downstream.

4. If the event is not serious: It is managed medically as appropriate, the information is recorded in the participant's study binder, transcribed onto the adverse event CRF, submitted to the Sponsor, and then followed until resolution.

5. If the event is determined to be an "unanticipated adverse device effect": The PI is required to report the event to the Sponsor and IRB by phone/email within 24 hours and then by written report within 5 business days after the PI first learns of the event.

The PI is responsible for reviewing and signing all completed adverse event forms for determination of serious events that require reports to the Sponsor and/or IRB.

The Investigator informs the Sponsor and IRB (when applicable) immediately upon knowledge of a serious adverse event. All information available on the event (hospital records, lab tests, discharge summaries, etc.) is forwarded to the Sponsor so they can determine whether the event is unexpected or associated, and the reporting outcome of the event. As additional information becomes available on the event, it should be forwarded to the Sponsor. This may be through the EDC or alternate means specified by the Sponsor.

SAE Reporting Timeline Summary

Event Type	Report To	Timeline	Regulatory Basis (Investigator obligation)
SAE (non-fatal, non-life-threatening)	Sponsor	Per protocol (typically ≤24h of site awareness)	ICH E6(R2) §4.11.1 — "immediately" + protocol-specified
Fatal or life-threatening SAE	Sponsor	Per protocol (typically initial ≤24h, full follow-up per protocol cascade)	ICH E6(R2) §4.11.1 + protocol
SAE / unanticipated problem (serious and unexpected, related)	IRB	Per IRB written policy (commonly ≤24h–10 working days depending on IRB)	21 CFR 56.108(b)(1); 45 CFR 46.103(b)(5); IRB SOP
All SAEs	Regulatory binder	File copies of all reports and follow-ups	ICH E6(R2) §4.11; §8.3.16–8.3.18

For reference (sponsor obligation, NOT investigator obligation): Sponsors must file IND safety reports with FDA within 15 calendar days of determining the event meets reporting criteria (or 7 calendar days for fatal/life-threatening unexpected suspected adverse reactions). Statutory basis: **21 CFR 312.32(c)(1)–(c)(2)**. Sites should ensure their reporting timeline upstream supports the sponsor's downstream 312.32 cadence.

5.1.1 Corrective and Preventive Action (CAPA) for Safety Events

When an SAE, AE pattern, or safety signal is identified, the PI and study team shall:

- 1. Assess root cause** — determine whether the event is related to the investigational product, study procedures, or external factors
- 2. Implement immediate corrective action** — address the specific event (e.g., dose modification, protocol amendment, additional monitoring)
- 3. Implement preventive action** — modify procedures to prevent recurrence across all current and future participants (e.g., additional training, enhanced screening, updated informed consent)
- 4. Document the CAPA** — record the root cause analysis, corrective actions taken, preventive measures implemented, and the responsible party in the study files
- 5. Follow up** — verify that corrective and preventive actions were effective; document verification and close out the CAPA when resolved
- 6. Report** — include CAPA details in protocol deviation reports, continuing review submissions, and monitoring visit documentation as applicable

5.2 Receipt of Adverse Events from Sponsor (IND Safety Reports)

- All IND safety reports (adverse events) received from the Sponsor will be forwarded to the PI.
- The PI will send copies to the IRB of record (as applicable).
- The CRC will place copies of the IND safety reports in the regulatory binder for that study.

5.3 Protocol Deviations

Any departure in study activity from the currently-approved protocol that typically does not have a significant effect on the participant's safety, rights or welfare, and/or the integrity of the resultant data will be reported according to applicable IRB policies.

- **Minor protocol deviations** will be tracked and reported to the IRB during continuing review (i.e., annual progress reports), unless the IRB's written policy requires more frequent reporting.
- **Major protocol deviations** (those that may affect participant safety, rights, welfare, or data integrity) are reported to the IRB **per the IRB's specified timeframe**, which **typically ranges from 5 to 10 working days of discovery** (e.g., Advarra and WCG generally require prompt reporting per their written policies — verify the timeline in the current IRB SOP at the time of the deviation).
- Major deviations are also reported to the Sponsor per the protocol's specified deviation reporting plan.
- The PI will determine if a deviation is major or minor.

Reports of protocol deviations are created and given to the PI for review and assessment before submission to the IRB. The report should include:

1. A detailed description of the incident
2. Indication as to whether the deviation placed the participant at any risk
3. A description of any changes to the protocol proposed in response to the deviation (if applicable)
4. Corrective and preventive action (CAPA) plans as necessary to ensure that such deviations will not occur in the future

The Research Director or designee with oversight of the research will be informed of major protocol deviations and ideally provided with an opportunity to review before the report is sent to the IRB.

ICH E6(R3) Note: Under E6(R3), protocol deviations should be evaluated within the context of the trial's overall quality management system. CtQ factors should be used to assess whether a deviation has a meaningful impact on participant safety or data integrity.

5.4 Protocol Exceptions

The research team will strive to follow the protocol as written; however, there are times when flexibility is sought (e.g., enrollment of a participant who does not meet the eligibility criteria).

- Requests for protocol exceptions must be discussed and approved by the PI, IRB, and Sponsor prior to initiation.
- The CRC will communicate and document approval or denials within the regulatory binder and/or participant-specific records.

5.5 Unanticipated Problems Involving Risk to Participants and Others

The PI must inform the Sponsor and IRB of:

- Unanticipated events related to the research that expose individuals other than the research participants (e.g., investigators, research assistants, students, the public, etc.) to potential risk
- Information that indicates a change to the risks or potential benefits of the research

Unanticipated problems, including those which may occur after the participant has completed or withdrawn from the study, will be reported to the IRB of record and the Sponsor.

The PI/CRC will report unanticipated problems involving risk to participants and others -- including but not limited to physical injury, psychological, economic, and social harm -- within designated time frames.

Examples include:

- Acts of God, power failures
- Breaches of confidentiality
- Loss of study data
- New information that might affect adversely the safety of the participants or the conduct of the clinical trial
- Any changes significantly affecting the conduct of the clinical trial or increasing risk to participants

5.6 Non-Compliance

If non-compliance is revealed, the PI and/or CRC will seek advice as necessary from the appropriate Research or Administrative Director and/or Office of Research Compliance.

Reports of non-compliance will be submitted to the IRB within 10 working days of discovery. The report must include:

- A complete description of the non-compliance
- The personnel involved
- A detailed description of the circumstances

Non-compliance includes:

- Failure to follow regulations, institutional policies governing human participant research, or requirements or determinations of the IRB
- Violations that: (1) create an increase in risks to participants, (2) adversely affect the rights, welfare, or safety of the research participants, or (3) adversely affect the scientific integrity of the study
- A pattern of non-compliance that, if allowed to continue, is likely to increase risk to participants, adversely affect the rights, welfare, and safety of research participants, or adversely affect the scientific integrity of the study

Since determinations of serious non-compliance, continuing non-compliance, or an unanticipated problem will be reported appropriately to external agencies and the Institutional Official, the Research or Administrative Director with oversight of the research will be informed of the non-compliance and ideally provided with an opportunity to review before the report is sent to the IRB.

6. Record Keeping

Records of all adverse events, protocol deviations, exceptions, unanticipated problems, and documentation of non-compliance should be kept for the same period as the other study documents. See SOP-007 (Data Management and Documentation) and [Institutional Policy #] on Maintenance, Storage, and Archiving of Human Participant Research Data.

7. References

- ICH E6(R2) Guidelines for Good Clinical Practice (Sections 1.2, 1.50, 4.11)
 - ICH E6(R3) Good Clinical Practice (current version -- introduces Quality by Design, Critical to Quality factors, and proportionality-based risk management)
 - 21 CFR 312.32 -- IND Safety Reporting
 - 21 CFR 312.64 -- Investigator Reports (annual progress reports to Sponsor and FDA)
 - 21 CFR Part 312 (IND regulations, general)
 - 21 CFR Part 812 (IDE regulations)
 - 45 CFR 46.103(b)(5) -- IRB reporting requirements for unanticipated problems
 - 45 CFR Part 46 (Common Rule)
 - [Institutional Policy #] -- Maintenance, Storage, and Archiving of Human Participant Research Data
 - Site Disaster Plan (see separate SOP)
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8. Revision History

Version	Date	Author	Description of Change
1.0	Initial genericized version		

Approved By: _____

Date: ____/____/____

Title: _____